

D776

Healthcare Leadership and Community Engagement

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A new hospital CEO, who previously ran a large urban system, wants to open a diabetes clinic in a rural county within 90 days because diabetes clinics worked in her last job. No local health data or resident input has been gathered. What should happen first?

- A. Open the clinic and survey patients after it launches.
- B. Extend the timeline to 180 days and then open it.
- C. Open the clinic, since it worked elsewhere.
- D. Conduct a community health needs assessment using local data and resident input to rank needs.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A health system planning a neighborhood wellness program has a report listing high obesity, few grocery stores, and low incomes. The neighborhood also has three active churches, a youth soccer league, and a retired nurses' group. The report was printed in full color. How should planning proceed under an asset-based approach?

- A. Partner with the churches, soccer league, and retired nurses as leaders and hosts of the program.
- B. Wait for a grocery store to open before doing anything.
- C. Design the program entirely around the deficit list.
- D. Bring in outside staff to run everything.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A clinic is relocating. The county commissioner controls the zoning vote but has shown little interest in the project. She has held office for 12 years. How should the project leader engage her?

- A. Engage heavily because long-serving officials always block projects.
- B. Keep her satisfied with brief, targeted updates and address her concerns before the vote.
- C. Manage closely with weekly meetings.
- D. Monitor only, since she is not interested.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

Residents living next to a planned clinic care deeply about parking and noise but have no formal say in approval. Which strategy fits?

- A.** Give them veto power.
- B.** Ignore them since they cannot block approval.
- C.** Keep them informed and give them channels to raise concerns.
- D.** Monitor only.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A public health department holds a town hall in its newly painted auditorium, presents a finished vaccination plan, and takes questions but makes no changes. It calls this “collaboration.” Where does it actually fall on the engagement continuum?

- A.** Collaborate
- B.** Empower
- C.** Consult
- D.** Inform

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A community board chooses which three health programs a hospital will fund, and the hospital agrees in advance to carry out its choices. What level of engagement is this?

- A.** Involve
- B.** Inform
- C.** Empower
- D.** Consult

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A nurse manager on a unit with a new coffee machine wants staff to embrace a fall-prevention initiative. She shares stories of patients harmed by falls, invites staff to design the approach, and coaches each nurse's growth. Which style is she using?

- A. Autocratic
- B. Transformational
- C. Transactional
- D. Laissez-faire

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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8. [TRANSFER — reversed stem / different variable]

A leader tells staff that anyone who meets hand-hygiene targets gets a bonus, and anyone who misses them gets a written warning. Which style is this?

- A. Transactional
- B. Laissez-faire
- C. Transformational
- D. Servant

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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9.

A health center wants to use a school's gym for evening flu clinics. The principal, who coaches the debate team, says no. When asked why, she explains she worries about security and cleanup costs. What is the best negotiation move?

- A. Ask her debate students to argue the case.
- B. Offer to use the gym only half the evenings.
- C. Push harder by stressing the clinic's importance.
- D. Offer to provide security staff and cover cleanup, meeting her concerns.

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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10. [TRANSFER — reversed stem / different variable]

Two partners are stuck: one insists a program run in the morning, the other in the evening. What should a leader do first?

- A.** Let the larger partner decide.
- B.** Ask each partner what need their preferred time meets.
- C.** Pick noon as a compromise.
- D.** Cancel the program.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A clinic in a town with a historic main street sees many missed appointments and poor diabetes control. Staff learn that most patients have no car, the nearest bus runs twice a day, and the only grocery store closed. What is the most effective leadership focus?

- A.** Add more appointment slots.
- B.** Clinical care alone always determines outcomes.
- C.** Partner on transport and food access, such as ride programs and a mobile market.
- D.** Assume patients are non-compliant.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A two-year grant funded a successful teen mental-health program led by one energetic coordinator who was recently featured on local TV. The grant ends in six months, and results have been reported as “number of events held.” What best supports lasting success?

- A.** Build partner ownership and diverse funding, and track outcomes such as reduced crisis visits.
- B.** Keep counting events to show activity.
- C.** Apply for the same grant again and change nothing.
- D.** Rely on the coordinator’s TV profile to attract new money.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A new hospital CEO, ~~who previously ran a large urban system,~~ wants to open a diabetes clinic in a rural county ~~within 90 days~~ because diabetes clinics worked in her last job. No local health data or resident input has been gathered. What should happen first?

Target word: first step **Decides:** No assessment has been done.

A	Inverted operation	Assessment should come before the intervention.
B	Wrong stem number	More time without assessment does not fix the gap.
C	Camouflage	Past success in a different community does not show local need.
D	Correct	Needs are identified before choosing an intervention.

2. Answer A Concept 2

A health system planning a neighborhood wellness program has a report listing high obesity, few grocery stores, and low incomes. The neighborhood also has three active churches, a youth soccer league, and a retired nurses' group. ~~The report was printed in full color.~~ How should planning proceed under an asset-based approach?

Target word: asset-based approach **Decides:** Build on existing community strengths.

A	Correct	Builds on existing strengths and relationships.
B	Dropped element	Ignores the assets already present.
C	Inverted operation	Deficit-only framing ignores local capacity.
D	Adjacent category	Service delivery by outsiders, not community development.

3. Answer B Concept 3

A clinic is relocating. The county commissioner controls the zoning vote but has shown little interest in the project. ~~She has held office for 12 years.~~ How should the project leader engage her?

Target word: power/interest quadrant **Decides:** High power, low interest.

A	Absolute language	Tenure does not decide engagement; the grid does.
B	Correct	High power, low interest calls for keeping satisfied.
C	Confusion set	That fits high-power, high-interest stakeholders.
D	Dropped element	Ignores her power over the vote.

4. Answer C Concept 3 · transfer

Residents living next to a planned clinic care deeply about parking and noise but have no formal say in approval. Which strategy fits?

Target word: low power, high interest **Decides:** Keep informed.

A	Absolute language	Overcorrects; they need information and a voice, not a veto.
B	Inverted operation	High interest makes them important to inform.
C	Correct	Low power, high interest: keep informed.
D	Confusion set	Fits low interest, low power.

5. Answer D Concept 4

A public health department holds a town hall ~~in its newly painted auditorium~~, presents a finished vaccination plan, and takes questions but makes no changes. It calls this “collaboration.” Where does it actually fall on the engagement continuum?

Target word: name the engagement level **Decides:** One-way presentation of a final plan.

A	Inverted operation	Collaboration shares decisions; none were shared.
B	Confusion set	Empower puts final decisions in community hands.
C	Adjacent category	Consulting gathers input that can shape decisions; nothing changed.
D	Correct	The plan was final; residents received information only.

6. Answer C Concept 4 • transfer

A community board chooses which three health programs a hospital will fund, and the hospital agrees in advance to carry out its choices. What level of engagement is this?

Target word: reverse: from description to level **Decides:** Community makes the final decision.

A	Adjacent category	Involve works with the community but the organization decides.
B	Inverted operation	The opposite end of the continuum.
C	Correct	Final decision-making rests with the community.
D	Confusion set	Consulting asks for feedback only.

7. Answer B Concept 5

A nurse manager on a unit ~~with a new coffee machine~~ wants staff to embrace a fall-prevention initiative. She shares stories of patients harmed by falls, invites staff to design the approach, and coaches each nurse’s growth. Which style is she using?

Target word: leadership style **Decides:** Vision, involvement, and development.

A	Inverted operation	Decides alone; she invites staff to design the approach.
B	Correct	Inspires through shared purpose and develops people.
C	Confusion set	Relies on rewards and corrections, not vision.
D	Adjacent category	Hands-off; she is actively engaged.

8. Answer A Concept 5 • transfer

A leader tells staff that anyone who meets hand-hygiene targets gets a bonus, and anyone who misses them gets a written warning. Which style is this?

Target word: reverse identification **Decides:** Rewards and penalties.

A	Correct	Exchanges rewards and penalties for performance.
B	Adjacent category	That style avoids direction.
C	Inverted operation	No vision or development, only incentives.
D	Confusion set	Centers serving staff needs, not rewards and warnings.

9. Answer D Concept 6

A health center wants to use a school's gym for evening flu clinics. The principal, ~~who coaches the debate team~~, says no. When asked why, she explains she worries about security and cleanup costs. What is the best negotiation move?

Target word: principled negotiation **Decides:** Address her interests: security and cleanup.

A	Camouflage	Her coaching role is irrelevant.
B	Adjacent category	Splits the difference without solving her concerns.
C	Inverted operation	Bargains on positions.
D	Correct	Targets her interests, not her position.

10. Answer B Concept 6 · transfer

Two partners are stuck: one insists a program run in the morning, the other in the evening. What should a leader do first?

Target word: move from positions to interests **Decides:** Ask why each wants the time.

A	Confusion set	Uses power, not interests.
B	Correct	Uncovering interests opens new options.
C	Adjacent category	Splitting the difference may serve neither interest.
D	Absolute language	Walks away before exploring options.

11. Answer C Concept 7

A clinic in a town ~~with a historic main street~~ sees many missed appointments and poor diabetes control. Staff learn that most patients have no car, the nearest bus runs twice a day, and the only grocery store closed. What is the most effective leadership focus?

Target word: root cause **Decides:** Transport and food access drive outcomes.

A	Dropped element	More slots do not help people who cannot get there.
B	Absolute language	Social conditions shape much of health.
C	Correct	Addresses social determinants driving outcomes.
D	Inverted operation	Blames patients for structural barriers.

12. Answer A Concept 8

A two-year grant funded a successful teen mental-health program led by one energetic coordinator ~~who was recently featured on local TV~~. The grant ends in six months, and results have been reported as “number of events held.” What best supports lasting success?

Target word: sustainability **Decides:** Shared ownership and outcome measures.

A	Correct	Shared capacity and outcome data sustain change.
B	Adjacent category	Activities are outputs, not outcomes.
C	Dropped element	Leaves the program dependent on one source.
D	Camouflage	One champion is a single point of failure.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owled.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	1	
Adjacent category	The right idea in the wrong bucket.	8	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	9	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	7	
Camouflage	Answers a stem fact that was never the question.	3	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Assess needs before acting	A community health needs assessment combines data and community voices to identify and rank needs before choosing interventions.	Launching the program leaders already favor and assessing afterward.	A leader eager to start a program with no local data or input.
2	Asset-based community development	Start from the community's existing strengths (people, groups, institutions) and build on them, rather than only cataloging deficits.	Treating residents as problems to fix and outsiders as the only source of solutions.	A plan that lists only what the community lacks.
3	Stakeholder power and interest	High-power, high-interest stakeholders are managed closely; high-power, low-interest are kept satisfied; low-power, high-interest are kept informed; low-power, low-interest are monitored.	Giving every stakeholder the same level of engagement, or ignoring powerful but quiet parties.	A stakeholder described by both influence and level of concern.
4	The engagement continuum	Engagement ranges from inform, consult, involve, collaborate, to empower; higher levels share more decision-making power with the community.	Calling a one-way announcement or a survey "collaboration."	Who actually makes the final decision.
5	Transformational vs. transactional leadership	Transformational leaders inspire change through shared vision and developing others; transactional leaders manage through rewards, rules, and corrective action.	Thinking transformational leadership means top-down orders for big change.	Vision and motivation vs. incentives and compliance.
6	Negotiate interests, not positions	Principled negotiation looks past stated positions to underlying interests and seeks options that meet both, knowing each side's best alternative.	Splitting the difference between positions or bargaining harder.	Two parties stuck on fixed demands.
7	Social determinants of health	Conditions where people live, work, and play (housing, transport, food, income, education) shape health outcomes more than clinical care alone.	Assuming more clinic services alone will fix poor outcomes.	Poor outcomes tied to transport, housing, or food access.
8	Sustained change needs local ownership and measures	Change lasts when community partners share ownership and capacity, and progress is tracked with specific, measurable goals.	Relying on one grant or one champion and measuring activities instead of outcomes.	A successful pilot about to lose its funding or leader.

Course scope (public description): Healthcare Leadership and Community Engagement focuses on leadership principles and how to apply them in real-world contexts. The course prepares students to analyze community needs and create change through community engagement. As leaders, the students' job is to engage in

collaborative approaches with an understanding that the overarching goal is sustained success. This course helps students develop their abilities to negotiate challenges, make decisions, and act to bring stakeholders together to create transformation within communities. There are no prerequisites for this course.